

How to Open a Brain Injury Residential Home in Colorado

An Assisted Living Residence certified as a Supported Living Program (SLP), funded through the Colorado HCBS Brain Injury (BI) waiver — a step-by-step startup and operations guide covering licensing, certification, staffing, and billing

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The Colorado model — read this first

Colorado serves adults with brain injury in a home that is licensed as an Assisted Living Residence and certified as a Supported Living Program. The license comes from the Colorado Department of Public Health and Environment (CDPHE), Health Facilities and Emergency Medical Services Division, under 6 CCR 1011-1 Chapter 7; the Supported Living Program (SLP) certification — also from CDPHE — is what lets the home serve people on the Home and Community-Based Services Brain Injury (BI) waiver and bill Medicaid for the specialized residential service under 10 CCR 2505-10 § 8.7550. There is one path: you become the licensed, certified provider yourself, enroll as a Health First Colorado (Medicaid) provider agency with the Department of Health Care Policy and Financing (HCPF), and bill the state directly against each member's prior authorization. This guide walks that path end to end — the business, the home, the two-step license-then-certify sequence, staffing, and the SLP per-diem — and points you to the manual and forms that satisfy the policies-and-procedures requirement on your application.

How to Use This Guide

This guide takes you from idea to an operating Colorado brain injury residential home. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Colorado providers — not legal, tax, or regulatory advice. Colorado's state fiscal year begins July 1, and license fees and waiver rates are updated annually; confirm every step, fee, and rate with CDPHE (Health Facilities and Emergency Medical Services Division) and HCPF before you rely on it, and recheck at the start of each fiscal year.

The one-paragraph version

Form your business and secure a home in a location where the residential use is zoned. Put professional liability insurance in place as the Health Care Availability Act requires. Submit a Letter of Intent to CDPHE and complete the Assisted Living Residence application — controlling-interest disclosures, insurance, organizing documents, floor plans, criminal history, and your policies and procedures compliant with Chapters 2 and 7 — then pass the FGI review, the Fire Marshal inspection, and the licensing survey to receive your ALR license. Add the Supported Living Program certification by Letter of Intent and a zero-deficiency SLP survey, which produces your Certification and Transmittal (C&T). Enroll as a Health First Colorado provider agency with HCPF under provider Specialty 680, attaching the C&T. Then accept member referrals from your regional Case Management Agency and bill the SLP per-diem (T2033 with the tier modifiers authorized on each member's prior authorization). Keep your Policy and Procedure Manual and Forms Packet current as your compliance backbone.

The Path at a Glance

Ten steps take you from idea to an operating, billing Colorado brain injury home:

1. **Form the business.** Register your entity with the Colorado Secretary of State; get an EIN; open a business bank account; keep your organizing documents.
2. **Insure and site the home.** Obtain Health Care Availability Act professional liability insurance, confirm zoning, and secure the physical home.
3. **Prepare the building.** Meet Chapter 7 physical-plant and, for new construction/renovation on or after July 1, 2020, the 2018 FGI standards; submit the ALR FGI Questionnaire.
4. **Write your policies.** Adopt the Policy & Procedure Manual and Forms Packet compliant with Chapters 2 and 7 — required with your application.
5. **Apply for the ALR license.** Submit the Letter of Intent and application through CDPHE's COHFI portal, with criminal history and all attachments.
6. **Pass licensing.** Clear the FGI review, the Fire Marshal inspection, and the CDPHE licensing survey to receive the ALR license.
7. **Add SLP certification.** Submit the SLP Letter of Intent and pass a zero-deficiency SLP survey; CDPHE issues your Certification & Transmittal (C&T).
8. **Enroll with Medicaid.** Enroll as a Health First Colorado provider agency with HCPF under Specialty 680, attaching the C&T; obtain your NPI.
9. **Accept referrals.** Take member referrals from your regional Case Management Agency against each member's prior authorization.
10. **Bill and operate.** Bill the SLP per-diem (T2033 + tier modifiers) and run the home to Chapter 7 and the SLP requirements.

Part 1 — What This Business Is

A Colorado brain injury residential home is a licensed Assisted Living Residence (ALR) — a home that provides three or more unrelated adults with room and board, personal services, protective oversight, social care, and 24-hour supervision — that is also certified by the state Medicaid agency as a Supported Living Program (SLP). The SLP certification is what allows the home to serve people on the Brain Injury waiver and to bill Medicaid for the specialized residential service.

This is a real-estate and operations business first: you are responsible for a licensed building, around-the-clock staffing, and the safety of adults who live under your roof and often have significant cognitive, behavioral, and physical support needs. It is more capital-intensive and slower to open than in-home care, but it builds a durable asset and a steady, census-based revenue stream once you are licensed, certified, and enrolled. Deciding this is the business you want — with both its weight and its rewards — is the real first step.

Demand for brain injury residential placements in Colorado is real and specialized. Members leaving hospitals or nursing facilities need a certified home that can provide 24-hour oversight, behavioral support, and cognitive supports — and there are far more members needing that setting than there are certified beds. What makes the SLP a defensible business is exactly what makes it demanding: the certification, the trained staff, the secure environment where needed, and the survey. Meeting those standards is what earns you referrals from Case Management Agencies and the trust of families placing a loved one.

What the Supported Living Program (SLP) provides

Under 10 CCR 2505-10 § 8.7550, the SLP is a specialized, certified assisted-living service for people with brain injury. Services include:

- 24-hour oversight and protective supervision
- Assessment, training, and supervision of self-care and independent living skills
- Medication management
- Behavioral management
- Cognitive supports

Who the home serves

The BI waiver serves individuals age 16 and older who have a brain injury of traumatic or acquired origin (occurring before the individual's 65th birthday), whose injury results in lasting physical, cognitive, emotional, or behavioral impairment of a non-progressive nature, and who meet a hospital or nursing-facility level of care (10 CCR 2505-10 §§ 8.7100, 8.7101.E). Members must be financially eligible for Health First Colorado (Medicaid) and enrolled in the BI waiver through a Case Management Agency.

Part 2 — Who Regulates You

It helps to see the three regulators as a chain rather than three separate hurdles. CDPHE decides whether your home is safe and your program qualifies; HCPF decides whether you can be paid and pays you; and the Case Management Agency decides which members come to you and at what authorized level. A gap with any one of them stalls the whole operation, so build relationships with all three early and keep each of them current on your status.

Three entities govern the home, and you will work with all three:

- **CDPHE — Health Facilities and Emergency Medical Services Division.** Licenses your Assisted Living Residence and certifies your Supported Living Program, and conducts the licensing and certification surveys.
- **HCPF — Department of Health Care Policy and Financing.** Colorado's Medicaid agency. Enrolls you as a provider agency, contracts with you, and pays your claims through the BI waiver.

- **Case Management Agencies (CMAs).** Determine member eligibility and level of care, authorize services through prior authorizations, and refer members to your home. There is at least one CMA in each defined service area.

The rules that apply to you

- **6 CCR 1011-1 Chapter 7** — Assisted Living Residences (your licensing and operating standard)
- **6 CCR 1011-1 Chapter 2** — General Licensure Standards
- **6 CCR 1011-1 Chapter 24** — Medication Administration (with C.R.S. 25-1.5-301 through 303)
- **10 CCR 2505-10 § 8.7000 series** — the HCBS Brain Injury waiver, including the SLP service at § 8.7550

Part 3 — Business & Insurance Setup

Before you apply, put the legal and financial foundation in place:

- **Form your business entity.** Register (for example, an LLC) with the Colorado Secretary of State and keep your articles of organization, operating agreement, and other organizing documents — CDPHE requires them with your application.
- **Obtain professional liability insurance.** Held in the name of the applicant as required by the Colorado Health Care Availability Act (C.R.S. 13-64-301 et seq.), listing CDPHE as a certificate holder.
- **Confirm zoning and secure the home.** Confirm local zoning approval for the residential use at your address, and lease or purchase the physical home.
- **Identify controlling interests.** Identify all persons and entities with a controlling interest in the home — you will disclose them on the application.

Part 4 — The Home: Physical Plant & Fire Safety

Your building must meet Chapter 7's physical-plant standards. For construction or renovation initiated on or after July 1, 2020, the 2018 Facilities Guidelines Institute (FGI) Guidelines for Design and Construction of Residential Health, Care, and Support Facilities apply; you submit an ALR FGI Questionnaire to request your on-site review.

Physical plant checklist

- Sleeping rooms: at least 100 square feet for single occupancy and 60 square feet per person for double occupancy; no more than two residents per sleeping room
- Each sleeping room has at least one window of 8 square feet with opening capability, and storage space (such as a closet) for each resident
- At least one full bathroom (toilet, hand-washing station, mirror) for every six residents; grab bars at tubs, showers, and toilets as needed by the population
- Accessibility for residents using auxiliary aids: accessible common and dining areas and doorways at least 32 inches wide; two means of egress if any resident uses an auxiliary aid
- Hot water does not exceed 120°F at resident-accessible taps; safe, potable water supply
- Oxygen stored and handled safely; hazardous substances labeled and locked away from confused residents with a safety data sheet available
- Fire safety: pass the local Fire Marshal inspection; disclose to residents whether the home has an automatic fire sprinkler system

If you will serve residents at risk of wandering (secure environment)

Many brain injury homes serve residents whose compromised safety awareness creates elopement risk. If you restrict free egress, you operate a “secure environment” and must meet the added requirements of Chapter 7, Part 25: enhanced disclosures and assessments (including a licensed-practitioner evaluation of wandering risk), a six-month re-assessment, secure-environment staff training, hazardous-item storage inaccessible to residents, monthly testing of securing equipment, and a secure fenced outdoor area at least six feet high. If you use egress-alert devices only to alert staff and redirect residents — not to lock or restrict egress — that is not, by itself, a secure environment.

Part 5 — Step 1: Get Your CDPHE Assisted Living Residence License

The ALR license is the foundation. It is applied for through CDPHE's online health facility portal (COHFI). Review CDPHE's “Initial Health Facility/Agency Licensing — Guidance and Instructions” and its document checklist first.

1. **Submit a Letter of Intent (LOI).** Request the Assisted Living Residence license application from CDPHE.
2. **Complete the application.** With all attachments: legal name and any other names used; controlling-interest persons; proof of professional liability insurance; organizing documents; the physical address (and records-storage address if different); a floor-plan map for each floor; and your policies and procedures, in full compliance with Chapters 2 and 7.
3. **Complete the criminal history record check.** An owner/applicant in Colorado more than three years requests a CBI fingerprint-based check with future-arrest notification; three years or less obtains an FBI fingerprint-based check through the CBI.
4. **Pay the non-refundable license fee.** ALR fees are based on licensed beds and posted on CDPHE's Facility Fees page by March 1 for a July 1 effective date; verify the current fee for your bed count.
5. **Submit the ALR FGI Questionnaire.** Request your on-site review, and schedule the Fire Marshal inspection.
6. **Pass the fitness review and licensing survey.** Once you pass and clear any plan of correction, CDPHE issues the Assisted Living Residence license.

Where your manual and forms fit

The application requires complete policies and procedures compliant with Chapters 2 and 7. Your Policy & Procedure Manual and the companion Forms Packet are built to satisfy this requirement and your ongoing compliance — have them ready before you apply.

Part 6 — Step 2: Add the SLP Certification & Enroll as a Medicaid Provider

Once you hold (or are adding to) an ALR license, you add the Supported Living Program certification and enroll with Medicaid so you can serve BI-waiver members and get paid.

1. **Submit an SLP Letter of Intent.** Ask CDPHE to add Supported Living Program services to your Assisted Living Residence license.
2. **Pass the initial SLP survey.** After a zero-deficiency survey, CDPHE creates a Certification & Transmittal (C&T) and uploads it to COHFI.
3. **Enroll with HCPF.** Enroll as a Health First Colorado provider agency, attaching your C&T to the Medicaid Provider Enrollment Application. The SLP is provider Specialty 680. Obtain your National Provider Identifier (NPI). HCPF may charge a provider enrollment fee.
4. **Accept referrals and bill.** Once enrolled and contracted, accept member referrals from Case Management Agencies and bill for the SLP service against each member's prior authorization.

A distinction to keep straight

A provider cannot be certified for both an Alternative Care Facility (Specialty 602) and a Mental Health Transitional Living Home (755). The SLP (680) and the Brain Injury Transitional Living Program (TLP, 682) are distinct services — know which one you are certified and enrolled for.

Part 7 — Staffing, Administrator & Training

Administrator

Your administrator must be at least 21, hold a high school diploma or equivalent, and have at least one year of experience supervising the delivery of personal care services including activities of daily living — or meet one of the alternative qualification pathways in Chapter 7, Part 6.3 (for example, a nursing or health-care license with experience, or a relevant degree with experience). The administrator must complete 40 hours of administrator training (Parts 6.5–6.7).

Direct-care staff

- Background checks before hire: name-based criminal history and a Colorado Adult Protective Services (CAPS) check
- Tuberculosis testing before direct resident contact; pre-employment health screening
- Initial orientation and role-specific training (Part 7.9), including dementia training and communication, behavioral-expression, and fall-prevention training
- Qualified Medication Administration Persons (QMAPs) for medication, meeting Chapter 24, with a designated QMAP supervisor who verifies competency by direct observation
- Brain-injury-specific competencies beyond the minimums: independent living skills training, behavioral management and de-escalation, cognitive supports, seizure recognition and response, and cognitive-communication strategies
- Sufficient staffing to provide the 24-hour oversight that defines the SLP service

Brain-injury care is genuinely different from general assisted-living care, and your staffing and training should reflect that. Residents may have unpredictable behavior, impaired safety awareness, seizure disorders, fatigue, and cognitive-communication differences that ordinary caregiver training does not prepare staff for. The homes that succeed treat de-escalation, cognitive supports, seizure response, and consistent routines as core competencies — trained, observed, and refreshed — not as extras, because that is what keeps residents safe and keeps the home out of trouble at survey.

Part 8 — Billing & Getting Paid: the SLP Per-Diem

You bill the Supported Living Program as a per-diem residential service (one unit equals one day) through the Brain Injury waiver, against each member's prior authorization. The Case Management Agency completes the prior authorization; you submit claims on the CMS 1500 claim form through the Provider Web Portal using your NPI. The service is billed with Place of Service 12 (Home) under provider Specialty 680.

SLP procedure code, acuity tiers & per-diem rates

Acuity Tier	Code + Modifiers	Per diem — Outside Denver	Per diem — Denver County
Tier 1	T2033 · U6	\$237.25	\$241.71
Tier 2	T2033 · U6, HB	\$277.39	\$283.96
Tier 3	T2033 · U6, HE	\$309.08	\$316.98
Tier 4	T2033 · U6, HK	\$370.29	\$380.92
Tier 5	T2033 · U6, HB, HE	\$407.77	\$420.18
Tier 6	T2033 · U6, HB, HK	\$453.04	\$467.89
Tier 7 (Negotiated)	T2033 · U6, HB, HK, SC	Facility-specific	Facility-specific

Unit: one unit = one day (per diem). Place of Service 12 (Home); provider Specialty 680. Tier 7 is a negotiated, facility-specific rate set by the Department using the member's acuity scores. The member's acuity tier and any Denver Regional Pricing rate are authorized on the prior authorization (do not add the HX modifier for SLP).

Confirm the current fiscal-year rate before you rely on it

The rates above are drawn from the HCPF Home and Community-Based Services — Brain Injury (BI) Waiver Rate Schedule and are illustrative. HCBS waiver rates are adjusted each state fiscal year (July 1) and can change within the year. Because a new state fiscal year begins each July 1, confirm the current per-diem for each acuity tier in the current HCPF BI waiver rate schedule and billing manual before you build your financial model.

The Community First Choice (CFC) transition

The BI waiver has been transitioning certain attendant and support services (such as personal care, CDASS, homemaker, remote supports, and home-delivered meals) to the Community First Choice (CFC) program, with members moving at their Continued Stay Review and those services leaving the waiver by the July 1, 2026 milestone. The residential Supported Living Program service (T2033, Specialty 680) is not among the services moving to CFC — it remains in the BI waiver. Still, confirm the current BI waiver service menu and the current per-diem rate for each acuity tier in the HCPF BI billing manual and fee schedule before relying on them, and recheck at the start of each fiscal year.

Part 9 — Timeline, Costs & Two Worked Examples

Rough timeline

Plan for several months from start to first billable day. The sequence is: form your entity and secure/prepare the home; submit the ALR Letter of Intent; complete the application and criminal history; FGI review, licensing survey, and Fire Marshal inspection; receive the ALR license; submit the SLP Letter of Intent; pass the SLP survey and receive your C&T; complete HCPF Medicaid provider enrollment; then accept referrals and bill. Survey scheduling and any plans of correction affect the total time; verify current timeframes with CDPHE.

Typical startup costs to budget

- The home (lease or purchase) and any FGI-compliant modifications, furnishings, and life-safety systems
- Professional liability insurance and business setup

- The bed-based CDPHE license fee (verify current) and any HCPF provider enrollment fee
- Staffing and training, including administrator training and QMAP certification
- Your Policy & Procedure Manual and Forms Packet

Worked example A — a 6-bed home in Colorado Springs

A single-family home is licensed as a 6-bed ALR and certified as an SLP. It serves BI-waiver members referred by the regional Case Management Agency, billing T2033 (U6 plus the tier modifiers authorized on each member's prior authorization) at the per-diem for each member's acuity tier. For illustration, using the rates above: a member authorized at Tier 3 (Outside Denver) generates \$309.08 per day; at a full census of six members near that tier, that is roughly \$1,854 per day, or about \$56,000 across a 30-day month, before expenses. Actual revenue depends on each member's authorized tier and the home's census; the home budgets 24-hour staffing to meet the SLP oversight requirement.

Worked example B — a home in the Denver metro

A similar home operating within the City and County of Denver may qualify for Denver Regional Pricing, which raises the approved SLP per-diem — for example, Tier 3 is \$316.98 per day in Denver County versus \$309.08 outside it. That higher rate is built into the prior authorization rather than added as a claim modifier. Otherwise the licensing, certification, staffing, and billing steps are identical. Confirm current Denver Regional Pricing in the HCPF Denver Regional Pricing appendix.

Part 10 — Common Mistakes to Avoid

- **Skipping the two-step sequence.** You must hold the ALR license before you can add the SLP certification and enroll with Medicaid — you cannot serve BI-waiver members or bill until all three are in place.
- **Serving a resident before written licensure.** Do not admit BI-waiver members until you hold the license, the SLP certification, and Medicaid enrollment.
- **A generic policy manual.** The application requires policies and procedures specific to Chapters 2 and 7; a borrowed or generic manual fails the survey.
- **Under-building the secure environment.** If you restrict egress you trigger Chapter 7, Part 25 in full — assessments, training, equipment testing, and the fenced outdoor area; do not restrict egress without meeting them.
- **Under-staffing the 24-hour oversight.** The SLP is defined by around-the-clock oversight; thin staffing shows up immediately in resident safety and survey findings.
- **Building a model on stale rates.** Rates change every July 1; confirm the current per-diem for each acuity tier before you commit.

Part 11 — Staying Certified & Growing

Once you are licensed, certified, and enrolled, staying that way is the ongoing work: keep your Policy & Procedure Manual and Forms Packet current and lived-in, maintain your insurance and QMAP coverage, complete required staff training on schedule, document resident care and incidents thoroughly, and prepare for your renewal and complaint surveys rather than reacting to them. When CDPHE amends Chapter 7 or HCPF updates the BI waiver rules or rates, update your policies and your financial model and re-train staff.

Growth usually means adding beds or a second licensed home, each separately licensed, certified, and surveyed, or building relationships with more Case Management Agencies so your beds stay filled with appropriately authorized members. Grow only as fast as you can staff and supervise well — in this field, quality and compliance are the reputation that earns you the next referral.

Part 12 — Your First 90 Days as a New SLP Home

Your first ninety days set the habits every later survey will examine. The home is new, the staff are new to each other, and the first members are settling in — so the routines you build now, done correctly, simply become how the home runs.

- **Make the manual live in practice.** Staff should be able to describe and follow the procedures they carry out — medication under Chapter 24, incident reporting, resident rights, and the SLP oversight — not just point to the binder.
- **Document from day one.** Keep resident records, the medication administration record, assessments, and incident logs accurate and current; a survey looks first at your documentation.
- **Confirm each member's authorization.** Bill only against a current prior authorization at the member's authorized acuity tier, and reconcile your claims against your census.
- **Run and record your drills.** Fire drills across shifts, and — if you operate a secure environment — the monthly securing-equipment tests and the added Part 25 requirements.
- **Watch staffing against oversight.** The SLP is defined by 24-hour oversight; keep your schedule matched to the members' needs and never run a shift short.

The compliance backbone

Two companion resources carry the documentation load: a Policy & Procedure Manual built to Chapter 7 and the SLP requirements that satisfies the policies-and-procedures item on your application, and a Forms Packet containing

the forms your policies reference — assessments, the medication administration record, competency tools, notices, and more. Together they give you the backbone a CDPHE survey expects.

Who to Call & What to Ask

For...	Who to contact
Forming your business	Colorado Secretary of State, Business Division (sos.state.co.us). Ask: how do I register my entity, and what are the ongoing filing requirements?
Licensing your home (ALR)	CDPHE, Health Facilities & Emergency Medical Services Division, via the COHFI portal (cdphe.colorado.gov). Ask: what does a complete Assisted Living Residence application require, and what are the current bed-based fees and timeframes?
The building & FGI review	CDPHE Health Facilities — ALR FGI Questionnaire and on-site review. Ask: which FGI standards apply to my construction date, and what does the on-site review cover?
SLP certification	CDPHE Health Facilities & Emergency Medical Services Division. Ask: how do I add the Supported Living Program to my ALR license, and what does the SLP survey require for a zero-deficiency result?
Medicaid enrollment & billing	HCPF, Department of Health Care Policy and Financing — provider enrollment (hcpf.colorado.gov). Ask: how do I enroll as a provider agency under Specialty 680, and where is the current BI waiver rate schedule and billing manual?
Member referrals & prior authorization	Your regional Case Management Agency (CMA). Ask: how are members referred and their acuity tier and prior authorization set, and how do I get on your referral list?
Background checks	Colorado Bureau of Investigation (CBI) for fingerprint-based checks; the Colorado Adult Protective Services (CAPS) check for staff. Ask: which check applies to my situation and how do I submit it?
Medication (QMAP)	CDPHE Medication Administration Program, under 6 CCR 1011-1 Chapter 24. Ask: how do my staff become QMAPs and how is competency verified?
Fire & life safety	Your local Fire Marshal and building department. Ask: what fire and life-safety approvals does my home need, and how do I schedule the inspection?

Key Terms

- **Assisted Living Residence (ALR):** the CDPHE license for a home serving three or more unrelated adults with room, board, personal services, and 24-hour supervision, under 6 CCR 1011-1 Chapter 7.
- **Supported Living Program (SLP):** the CDPHE certification and Medicaid service (10 CCR 2505-10 § 8.7550) that lets an ALR serve BI-waiver members and bill the specialized residential per-diem; provider Specialty 680.
- **BI waiver:** the HCBS Brain Injury waiver funding community-based services for eligible members with brain injury.
- **CDPHE:** Colorado Department of Public Health and Environment — licenses the ALR and certifies the SLP.
- **HCPF:** Department of Health Care Policy and Financing — Colorado’s Medicaid agency; enrolls providers and pays claims.
- **CMA:** Case Management Agency — determines eligibility and level of care, sets the prior authorization, and refers members.
- **C&T:** Certification & Transmittal — CDPHE’s document confirming SLP certification, attached to your Medicaid enrollment.
- **QMAP:** Qualified Medication Administration Person — staff authorized to administer medication under Chapter 24.
- **FGI:** Facilities Guidelines Institute — the 2018 design and construction standards applying to newer construction or renovation.
- **Per-diem / acuity tier:** the SLP is paid one unit per day at a rate set by the member’s acuity tier (T2033 with tier modifiers).
- **Secure environment:** a home that restricts free egress, triggering the added requirements of Chapter 7, Part 25.
- **CFC:** Community First Choice — the program certain attendant services are moving to; the residential SLP remains in the BI waiver.

Core Rules & References

- 6 CCR 1011-1 Chapter 7 — Assisted Living Residences (CDPHE, Health Facilities & Emergency Medical Services Division)
- 6 CCR 1011-1 Chapter 2 — General Licensure Standards; Chapter 24 — Medication Administration (with C.R.S. 25-1.5-301 through 303)
- 10 CCR 2505-10 § 8.7000 series — HCBS Brain Injury (BI) waiver, including the Supported Living Program at § 8.7550; §§ 8.7100, 8.7101.E — eligibility and level of care
- C.R.S. 13-64-301 et seq. — Colorado Health Care Availability Act (professional liability insurance)
- HCPF HCBS — Brain Injury (BI) Waiver Rate Schedule and BI billing manual (T2033, Specialty 680) — confirm the current fiscal-year figures

This guide is a general educational resource for prospective Colorado brain injury residential home providers. It is not legal, tax, or licensing advice, and requirements, fees, forms, rates, and contacts change. Verify every step, figure, and form with the agency named — the Colorado Secretary of State, CDPHE (Health Facilities & Emergency Medical Services Division), HCPF, your regional Case Management Agency, and your local Fire Marshal — and recheck at the start of each state fiscal year (July 1). Provisions cited reflect Colorado law and CDPHE and HCPF rules at the time of writing.